



MOTHER BABY UNIT PROCEDURE	
Safety Planning – Mother Baby Unit (MBU)	
Scope (Staff):	Mental health staff working in WNHS
Scope (Area):	Mother Baby Unit (MBU)

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1. Aim

The purpose of this guideline is to provide direction and information to all mental health clinical staff of the Women's Health, Genetics and Mental Health Directorate (WHGMHD) regarding the assessment and management of risk to patients, consistent with the [WA Health Mandatory Policy MP 0181/24 Safety Planning for Mental Health Consumers Policy](#) and the [Chief Psychiatrist's Guidelines for the Sexual Safety of Consumers of Mental Health Service in Western Australia](#).

2. Key Points

- The MBU risk management approach is in line with [Women's and Perinatal Mental Health Referral and Management Guideline](#) and follows principles based on best practice.

- MBU is an authorised but not a secure unit. Patients may be detained on the basis of clinical examination by the treating or on-call psychiatrist under the [Mental Health Act 2014](#) in the setting of risk to themselves, baby or others.
- Patients will have ongoing regular review by the psychiatrist to assess change in risk so that the patient is managed in the least restrictive way.
- Risk can vary over time and staff need to respond promptly to the changing assessment of risk.
- Risk can never be fully eliminated: environmental risks require consideration as part of the risk management plan.
- Developing a therapeutic relationship with patients and their families is key to assessing and lowering risk.
- All staff will undertake the appropriate specialist training to safely manage clinical deterioration in mental health as per the WNHS training matrix.

3. Process

3.1. Environmental factors

Supervised Access

A number of areas within the MBU have been identified as those where risk management is more challenging. Therefore, access to these areas will be supervised only:

- Air locks - A staff member must be aware when a patient is passing through an airlock.
- Group room - A staff member must maintain visual contact with patients in the group room.
- Ward garden - Access to the ward garden is controlled by a security swipe card. Any patient in the ward garden must be under visual contact by a staff member. Involuntary patients must be on one-to-one special observations whilst in the garden. The proximity of staff to patient must be specified by medical staff and documented in the patient's healthcare record.
- Laundry – swipe access only.

Supervision of patients is to be undertaken by nursing, medical and allied health professionals (not PCAs and security staff).

Restricted Access

Patients are not allowed in areas for staff access only. These areas include:

- nursing office
- staff room
- storeroom
- sluice room

Equipment

- Equipment (knives, scissors, therapy items etc) that can potentially harm patients, staff or others must be stored in lockable drawers or cupboards in a non - patient area.
- Staff must be cognisant of potential risks with everyday objects that can be found in the communal areas on the MBU. These include kitchen items such as crockery and metal cutlery. Some patients may require supervision in these areas if there are risks identified.
- Patients must be supervised by nursing, medical or allied health professionals, when using any potentially dangerous equipment.

Room Allocation

- Room allocation must be appropriate to the level of risk – patients on higher observation levels are to be managed in rooms 1 to 4.

Ligature Points

- Ligature points occur throughout the MBU. Staff will be made aware of potential ligature points at orientation and must make themselves aware of any of any changes or updates to this.
- Assessment will be made as to level of risk of self-harm and supervision required to mitigate the risk.
- Patients identified as high risk of harm will be assessed as to whether potential risk items (including, but not limited to, belts, bras, shoelaces, handbag straps, hoody cords) can remain with them.
- If a patient is identified as high risk of harm to self or infant, assessment will be made as to whether the cot is removed from the room.
- Any assessment, management plan and rationale will be recorded in the healthcare record. Any change to risk needs to be updated and recorded when it occurs.

3.2. Prior to admission

- At the point of triage, all patients will be advised by the CNM/CNC of the requirements of a search process on admission along with other points of information such as valuables, clothing requirements etc.

3.3. On Admission

- As per MBU Search procedure all patients' items will be checked on admission to reduce the risk of harm. Further searches will be conducted as set out in the [MBU Search Procedure](#).
- A general review of the ward environment will be conducted daily – risk is to be balanced between the ward milieu and individual patient risk as outlined in [MBU Search Procedure](#).
- During the admission process all patients are assessed for potential risk to themselves (both physically and mentally) and others, including their babies.

- The immediate risk of harm, including physical or emotional neglect, to the baby will be assessed on admission and the level of infant observations implemented based on this. Any concerns about threats, abuse or neglect of the infant by the mother or family of the infant will be discussed with the social worker allocated to the MBU.
- A mental state risk assessment is completed during the admission process including a mental health risk assessment and management plan. The Inpatient [Risk Assessment and Management Plan \(SMHMR905\)](#) Form should be completed by the admitting doctor.
- A physical assessment is conducted within 12 hours of admission to the MBU.
- Patients admitted to the MBU are required to have a falls risk assessment within six hours of admission using the [NMAHS Mental Health Falls Risk Management Tool \(FRMT\) PMR 100](#).
- All patients are assessed for risk of falls or pressure injuries. If indicated, a pressure assessment will be completed.
- The Consultant Psychiatrist/doctor who admits the patient determines the initial MBU category observation rating for the patient and the mother/infant dyad.
- Category observation rating will determine the supervision required and depends on the degree of risk established during the assessment process. Category observation ratings are documented on the whiteboard in the office, on the handover sheet, and in the patients' healthcare record.

3.4. Clinical Observation Requirements

Decisions regarding clinical observation requirements are influenced by the severity of the patient's illness, their level of insight, compliance with treatment, vulnerability, degree of impulsiveness and the presence of active suicidal ideation (refer to [WNHS Recognising and Responding to Acute Clinical Deterioration \(Physiological and Mental Health\) Policy](#)). Clinical observation levels should be determined by medical staff for every patient in consultation with nursing staff and reviewed as required for observation and ongoing assessment of the patient's level of risk.

3.5. Minimum Observation Category Requirements

- Check the observation level in the medical record and discuss any documented restrictions with the patient.
- Check and document that the patient is present in the ward area at a minimum of one hourly intervals or has permission to leave the ward.
- Record the observations checks on the patient and document the time on Appendix 1: Mother and Baby Visual Observations Checklist accurately and contemporaneously
- Check that the patient is present at mealtimes and when the ward is secured for the night and document as required (Refer to [Specials and Clinical Observation Report MR285.15](#)).

- Notify the nurse in charge of the shift immediately if the patient cannot be accounted for at agreed/specified time. Refer to the 'Absent without Leave' section of the [Applying the Mental Health Act 2014 in WNHS](#) Clinical Practice Guideline.
- Ensure that an update is provided on the whereabouts and well-being of every patient in their care as part of the nursing communication handover process at the end of each shift.
- Respiration rates must be recorded at a minimum of 1-hour intervals throughout the night when patients are asleep.

3.6. During the admission

- Reviews of risk assessment occur during clinical contact on a daily basis and must be documented daily in the integrated progress notes, and observation levels changed as required on the journey board and handover sheet.
- Nursing staff can increase patient visual observations if risk increases. Only the treating or on-call psychiatrist, Senior Medical Officer or Registrar can decrease visual observations, in consultation with the multidisciplinary team.
- The Shift Co-ordinator must inform the treating/on-call psychiatrist immediately of any escalation in risk or significant risk issue.
- Observation levels will be considered as part of the routine clinical review, any changes need to be documented in the medical notes and on the patient board. Consideration of review over the weekend should be made if there are changes in clinical presentation.
- If risk changes, a new Mental Health Risk Assessment and Management Plan (RAMP [SMHMR905](#)) must be completed and communicated to the treating/on-call psychiatrist as well as the rest of the team.
- The treating/on-call psychiatrist is to formulate an appropriate action plan which is documented and then communicated to multidisciplinary team by the psychiatrist.
- If there are still ongoing concerns about risk issues, the Shift Coordinator must keep the treating/on-call psychiatrist as well as the Hospital Clinical Manager informed on a shift-by-shift basis.

3.7. Care and management of higher risk patients

There will be instances where patients are identified as requiring higher levels of nursing observation due to risks. This may include (but is not limited to):

- risk of self-harm or suicide
- risk of harm to others
- impaired cognitive awareness and judgement such as delirium or intoxication or withdrawal from substances, risk of absconding
- extreme state of excitement or mania
- extreme sexual vulnerability.

3.8. One to One Nursing (Special)

- The decision for a 1-1 nurse special is determined by the level of risk as identified on the Risk Assessment and Management Plan.
- The treating psychiatrist will, in collaboration with Senior Nursing staff/shift coordinator, make the decision to implement a 1-1 nurse special.
- After hours, the shift coordinator may implement a 1-1 nurse special. The treating psychiatrist and the Hospital Clinical Manager are informed of this decision as soon as possible.
- It is the responsibility of the CNC/M or the shift coordinator to ensure adequate staffing in the case of 1-1 nurse special. The Hospital Clinical Manager is to be advised of all staffing issues.
- If a patient is placed by nursing staff on a special the consultant must be notified and a time frame for review arranged as soon as is practicable.
- The treating/on-call psychiatrist must review any patient on a 1-1 nurse special daily.
- Only the Consultant Psychiatrist can cease the 1-1 nurse special.
- The proximity of nurse to patient must be specified in the healthcare record and observation chart. For example, within arm's length from patient at all times which includes the toilet and shower or may be in close proximity and allow privacy in the toilet and shower.
- Any deviations outside of this must be recorded in the healthcare record and authorised by the treating psychiatrist.

A patient on one-to-one nursing:

- **must** remain within arm's reach at all times unless otherwise specified**
- **must not** be left unattended for any reason and be particularly closely observed during staff handover, staff meal breaks, ward disturbances, and visits to the toilet
- may attend ward programs within the unit as is deemed appropriate by the treating team, accompanied by the one-to-one nurse special

Staff must wear a duress alarm whilst undertaking a special.

The allocated one-to-one nurse is responsible for:

- a comprehensive evaluation of the nursing care plan at the end of each shift
- giving a comprehensive verbal handover using ISoBAR format to the oncoming one-to-one nurse including:
 - present mental and physical state
 - identified problems
 - allocated nursing actions.

Any changes in the patient's mental or physical state that relate to increased risk factors must be reported immediately to the shift coordinator, who will inform the patient's treating psychiatrist.

All patients on one-to-one nursing must have observations (including respirations if the patient is asleep or lying down resting) recorded every 15 minutes.

Any deviations outside of the above must be recorded in the healthcare record and authorised by the treating psychiatrist.

3.9. General Management of 1-1-nursing

- A new observation form must be commenced at the beginning of each shift for all patients on observation levels 2-4. The form must **not** be pre-populated with times, and visual checks are **not** to be undertaken at set times i.e. on the hour, quarter past and half past the hour.
- Consideration must be given to the appropriate nurse-patient allocation by the shift co-ordinator. Nursing staff must work within their scope of practice. The co-ordinator must take this into account during allocation. A second nurse will be identified to provide support, breaks and medication runs.
- The allocated nurse is to engage in meaningful activity with the patient during their shift. Diversional tasks such as arts and crafts, puzzles, baking, washing clothes, gardening ought to be considered as well as individual CBT strategies and mindfulness.
- If safe, the patient will be supported by nursing staff to engage with, and care for, their baby to maintain and develop the mother-infant bond.
- Patients are at increased risk during busy periods, e.g., meals, handover/shift change. Increased vigilance is required during these times.
- Suicide risk must be assessed each shift by nursing staff, making a record in the healthcare record detailing thoughts, plans, intent, mood and behaviour. Staff will also consider and explore protective factors with the patient.
- Regular medication and PRN will be considered and prescribed by the treating or duty psychiatrist to treat the underlying illness and help alleviate distress. All PRN medications administered must be recorded in the patient's healthcare record with a reason for administration.
- Extended periods of care on 1-1 nursing will require the multidisciplinary team (MDT) to assess the appropriateness of the patient's care continuing at the MBU. Factors to be considered when assessing this includes (but is not limited to) the capacity of the MBU and staff to maintain a safe environment, the patient's engagement with the programme and ability to provide care for their baby.

- If a patient requires an extended period of time on high level of observation, debrief for staff can be arranged. Staff are to be encouraged to attend regular group supervision available to all working at the MBU.

All staff must adhere to their professional clinical judgement – if there are concerns raised by any member of the team staff should always err on the side of caution and raise observation levels until an assessment of risk is undertaken by the treating psychiatrist and the MDT.

4. Care and Management of Suicidality

Staff should refer to the [Safety Planning Procedures for Mental Health Consumers](#)

Assessment of Suicide Risk

- Enquiry to ascertain possible suicide risk is part of the medical and nursing assessment for every patient on admission to the MBU.
- Assessment of suicidal or self-harming ideation, volition, plan and intent occurs on admission and regularly throughout admission. This should be documented in the healthcare record and communicated to clinical staff at every handover.
- Observation levels must be identified and documented in the Statewide Standardised Clinical Documentation (SSCD) on admission at this initial assessment.

Notification of Suicide Risk

- If any member of the multidisciplinary team receives information from the patient or others suggesting suicidal risk, they should personally inform the patient's treating doctor and senior nurse on duty without delay and document in the patient's clinical case record.

Management of Suicide Risk

- Management of suicide risk will be determined based on the clinical circumstances of the individual patient.
- A comprehensive care plan will be formulated that incorporates risk and is clearly documented in the patient's notes.
- Individual preventative strategies will be determined by the clinical team in consultation with the patient, their partner or PSP (if appropriate) and included in the care plan.
- Risk must be reviewed at least every 24 hours in consultation with medical staff.
- Changes in observation level must be documented in the patient's notes.
- Nurse-initiated increase in observation levels must be documented in the patient's healthcare record, together with reasons and rationale for change.

An updated risk assessment is to be completed outlining change in risk presentation.

Out of hours, the on-call consultant psychiatrist must be notified as soon as is practicable and agreed a time frame for review.

Considerations for managing suicidality

- Individual risk factors well as protective factors must be identified and documented in the patient's healthcare record.
- Discussions with the patient's partner/PSP regarding suicidality, risk and protective factors as well as the patient's management plan must be documented.
- To maintain the patient's safety the management plan must be drawn up in consultation with the patient. If they are unwilling to engage in the process this must be documented in the patient's healthcare record. Staff must continue to try to engage the patient in their management plan.
- In consultation with the treating team, items that are not routinely removed such as laces, belts and bras are to be considered for removal from patients. The decision regarding removal is to be recorded in the healthcare record. Any items retained (or returned) to patients must also be recorded in the healthcare record.
- Any deviations outside of this must be recorded in the healthcare record and authorised by the treating psychiatrist
- Granting of leave is to be considered along with an assessment of risk and the patients' observation levels and recorded in the healthcare record by the treating psychiatrist.
- Refer to [MBU Escort Procedures](#) in regard to maintaining a level of supervision when escorting patients outside of the unit.

4.1. On discharge/transfer

- Prior to discharge/transfer risk issues must be reassessed and documented in the healthcare record. This information will be provided to community services and any concerns highlighted. Any escalation of risk at this transition period will be clearly and promptly communicated to the most appropriate services able to manage care.
- Risk management at this point ought to include discussions with the family and/or personal support person about discharge/follow up plans as well as crisis plans, ongoing medication management including dispensing protocols. This information will be provided in the discharge summary.
- On discharge/transfer an updated RAMP will be included on webPSOLIS.

Related policies and procedures

Related policies

[Mental Health Act 2014](#)

[WA Health Mandatory Policy MP 0181/24 Safety Planning for Mental Health Consumers Policy](#)

[Principles and Best Practice for the Care of People Who May Be Suicidal](#)

[Chief Psychiatrist's Guidelines for the Sexual Safety of Consumers of Mental Health Services in Western Australia](#)

Related WNHS procedures and guidelines

[Applying the MHA 2014 at WNHS Guideline](#)

[Women's and Perinatal Mental Health Referral and Management Guideline](#)

[MBU Search Procedure](#)

[MBU Escort Procedures](#)

Useful resources

[Integrated Progress Notes MR250](#)

[Statewide Standardised Clinical Documentation \(SSCD\) Forms](#)

[A national framework for recovery-oriented mental health services: guide for practitioners and providers](#)

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NSQHS Standards Applicable:	 Std 1: Clinical Governance  Std 5: Comprehensive Care		 Std 6: Communicating for Safety  Std 8: Recognising and Responding to Acute Deterioration		
Chief Psychiatrist's Clinical Standards of Care		Risk Assessment and Management			
Printed or personally saved electronic copies of this document are considered uncontrolled					

Version history

<u>Version notes</u>	<u>Date</u>
First published version. Merge of Psychological Medicine and MBU guidelines Title- Clinical Practice Guideline: Risk Management- Mental Health.	August 2017
Risk Management- Mental Health rescinded	August 2021
New version: WNHS Procedure Risk Management-MBU. Procedure formatting and content adapted to template	August 2021
Three yearly review- amendments include: • title change to MBU Safety Planning from MBU Risk Management to comply with WA Health Mandatory Policy MP 0181/24 Safety Planning for Mental Health Consumers Policy • MP0074/17 replaced with MP0181/24 • amended hyperlinks • minor grammatical changes and rewording • minor formatting changes including removal of National Standards for Mental Health Services and replaced with Chief Psychiatrist's Clinical Standards of Care to align with NMHS MH guidelines Added: • Chief Psychiatrist's Guidelines for the Sexual Safety of Consumers of Mental Health Service in Western Australia. • stable links (where available) • laundry- swipe access only • staff will be made aware of potential ligature points at orientation and must make themselves aware of any of any changes or updates to this. • risk of harm to self or infant	4 August 2025

• risk assessment for should be completed by the admitting doctor. • falls risk assessment within six hours • A national framework for recovery-oriented mental health services: guide for practitioners and providers (in useful resources) Changed: • Patient notes/medical file to healthcare record • Hospital Nurse Manager to Hospital Clinical Manager Removed: • paragraphs related to determination of risk and observations • (rooms 3 & 4 to be considered if the risk is lower at night when sleeping). This may mean moving rooms during a patient's admission. • (related to discussion with social worker) on admission and in the weekly multidisciplinary meeting • RAMP removed from notification of suicide section; addressed in admission section Endorsed by Mental Health CPOC 4 August 2025	
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APPENDIX 1: MOTHER & BABY VISUAL OBSERVATIONS CHECKLIST

This is to confirm the whereabouts, safety and well-being of inpatients within the ward / area. Visual observations must be recorded at a **minimum of hourly** for all patients. **Respiration rates must be accurately counted and recorded when patients are asleep and when otherwise clinically indicated.** Clinical Observations and routine vital signs monitoring must continue **in addition** to these hourly visual observations. Care must be escalated if clinical deterioration is assessed in either physical or mental state.

Please do not discard this checklist.

Date:	ROOM	OBS	0700	0800	0900	1000	1100	1200	1300	1400	1500	1600	1700	1800	1900	2000	2100	2200	2300	2400	0100	0200	0300	0400	0500	0600
Mother:	1																									
Baby:																										
Mother:	2																									
Baby:																										
Mother:	3																									
Baby:																										
Mother:	4																									
Baby:																										
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Baby:																										
Mother:	6																									
Baby:																										
Mother:	7																									
Baby:																										
Mother:	8																									
Baby:																										
Staff Initials																										
Environment Check		Staff signature AM:												Staff signature PM:												

Legend: S= Sleep (include resps); A= Awake; M= Missing; O= Out; D/L= Daily Leave; √ = present on unit R=restless